

Healthy People 2030 (in 2026), Access to Health Services

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About This Literature Summary

This summary of the literature on Access to Health Services as a social determinant of health is a narrowly defined examination that is not intended to be exhaustive and may not address all dimensions of the issue. Please note: The terminology used in each summary is consistent with the respective references. For additional information on cross-cutting topics, please see the [Access to Primary Care](#) literature summary.

Related Objectives (4)

Here's a snapshot of the objectives related to topics covered in this literature summary. [Browse all objectives](#) .

- [Increase the proportion of adolescents who had a preventive health care visit in the past year — AH-01](#)
- [Increase the proportion of people with health insurance — AHS-01](#)
- [Increase the proportion of people with dental insurance — AHS-02](#)
- [Increase the proportion of adults who get recommended evidence-based preventive health care — AHS-08](#)

Related Evidence-Based Resources (5)

Here's a snapshot of the evidence-based resources related to topics covered in this literature summary. [Browse all evidence-based resources](#) .

- [Breast Cancer: Screening](#)
- [Cervical Cancer: Screening](#)
- [Colorectal Cancer: Screening](#)
- [Improving Access to Oral Health Care for Vulnerable and Underserved Populations](#)



Healthy People 2030 organizes the social determinants of health into 5 domains:

1. Economic Stability
2. Education Access and Quality
3. Health Care Access and Quality
4. Neighborhood and Built Environment

5. Social and Community Context

Literature Summary

The National Academies of Sciences, Engineering, and Medicine (formerly known as the Institute of Medicine) define access to health care as the “timely use of personal health services to achieve the best possible health outcomes.”¹ Many people face barriers that prevent or limit access to needed health care services, which may increase the risk of poor health outcomes and health disparities.² This summary will discuss barriers to health care such as lack of health insurance, poor access to transportation, and limited health care resources, with a special focus on how these barriers impact under-resourced communities.

Unequal distribution of health care coverage contributes to disparities in health.² Out-of-pocket medical care costs may lead individuals to delay or forgo needed care (such as doctor visits, dental care, and medications),³ and medical debt is common among both insured and uninsured individuals.^{3, 4} People with lower incomes are often uninsured,^{5, 6, 7, 8} and minority groups account for over half of the uninsured population.⁹

Lack of health insurance coverage may negatively affect health.^{9, 10} Uninsured adults are less likely to receive preventive services for chronic conditions such as diabetes, cancer, and cardiovascular disease.^{10, 11} Similarly, children without health insurance coverage are less likely to receive appropriate treatment for conditions like asthma or critical preventive services such as dental care, immunizations, and well-child visits that track developmental milestones.¹⁰

In contrast, studies show that having health insurance is associated with improved access to health services and better health monitoring.^{12, 13, 14} One study demonstrated that when previously uninsured adults ages 60 to 64 years became eligible for Medicare at age 65 years, their use of basic clinical services increased.¹³ Similarly, providing Medicaid coverage to previously uninsured adults significantly increased their chances of receiving a diabetes diagnosis and using diabetic medications.¹⁵ Medicaid coverage is also critical for enabling children with special health needs or chronic illnesses to access health services. The Children’s Health Insurance Program (CHIP) offers sole coverage for 41 percent of children with special health care needs.¹⁶ Many health care resources are more prevalent in communities where residents are well-insured,¹⁰ but the type of insurance individuals have may matter as well. Medicaid patients, for instance, experience access issues when living in areas where few physicians accept Medicaid due to its reduced reimbursement rate.^{14, 17, 18}

Health insurance alone cannot remove every barrier to care. Limited availability of health care resources is another barrier that may reduce access to health services and increase the risk of

poor health outcomes. ¹⁹ , ²⁰ For example, physician shortages may mean that patients experience longer wait times and delayed care. ¹⁸

Inconvenient or unreliable transportation can interfere with consistent access to health care, potentially contributing to negative health outcomes. ²¹ Research has shown that individuals from racial/ethnic minority groups who had an increased risk for severe illness from COVID-19 were more likely to lack transportation to health care services. ²² Transportation barriers and residential segregation are also associated with late-stage presentation of certain medical conditions (e.g., breast cancer). ²³ , ²⁴ , ²⁵

Expanding access to health services is an important step toward reducing health disparities. Affordable health insurance is part of the solution, but factors like economic, social, cultural, and geographic barriers to health care must also be considered, ²⁰ as must new strategies to increase the efficiency of health care delivery. ¹⁸ , ²⁶ , ²⁷ Further research is needed to better understand barriers to health care, and this additional evidence will facilitate public health efforts to address access to health services as a social determinant of health.

Citations

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